

Methodology Note

This methodology note describes the process behind the collection and analysis of speeches from the Norwegian Parliament. It is intended to provide a brief overview so that readers can understand how the data were collected, prepared, and analyzed. The complete syntax files are available in the GitHub repository.

The analysis of health-related speeches in the Norwegian Parliament consists of two syntax files. The first file contains the code used to collect the data. The speeches were gathered using the R package `stortingscrape`, developed by Martin Søyland. More information about the package is available here: <https://martigso.github.io/stortingscrape/>.

The data collection covers speeches published during a one-year period, from July 28, 2025, to July 28, 2026. A total of 15,012 speeches were collected during this period. These include speeches delivered by different parliamentary speakers representing several political parties.

The second syntax file contains the preparation, analysis, and visualization of the data. In this part of the project, I examine the occurrence of health-related terms across the parliamentary speeches. Rather than coding the speeches manually or using large language models to classify the material, I use a predefined list of Norwegian keywords. These keywords are used to identify speeches that refer to different dimensions of access to healthcare.

The dimensions include waiting times and capacity, geography and local services, financial barriers, patient rights and choice, primary care and continuity, and shortages of healthcare personnel and recruitment challenges. A speech may contain terms associated with more than one dimension and can therefore be included in several categories.

The speakers are also grouped according to political bloc. The left includes the Labour Party, the Socialist Left Party, and the Red Party. The center includes the Centre Party, the Liberal Party, the Christian Democratic Party, and the Green Party. The right includes the Conservative Party and the Progress Party. Government speakers are excluded from the comparisons between the political blocs because they speak from a different institutional position than ordinary parliamentary representatives.

Although the data collection covers a full year, I decided to include only speeches from 2026 in the main analysis. This was done to keep the results as current as possible. The collection script therefore covers a longer period, while the analytical script includes speeches published from January 1, 2026, onward.

The analysis is descriptive and examines how frequently the different dimensions of healthcare access appear in parliamentary speeches. It also compares how much attention the political blocs give to healthcare access and whether they emphasize different dimensions of the issue. The analysis measures political attention and the ways in which access to healthcare is discussed. It does not measure the actual quality or availability of healthcare services.

The keyword-based approach also has some limitations. A speech may discuss access to healthcare without using one of the predefined terms, while some keywords may occasionally appear in a different context. The results should therefore be understood as a systematic overview of patterns in parliamentary language rather than a complete classification of every discussion concerning access to healthcare.